Document Control

Title			
Protocol for Pain Management in Minor Injury Units			
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Directorate Emergency Services, Logistics and Resilience			**Department** Emergency Department
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Policy categories for Trust's internal website (Bob)	Tags for Trust's internal website (Bob)
MIU Standard Operating Procedures	None

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1. Purpose

This Protocol is for the use by staff employed by Northern Devon Healthcare Trust who have achieved the agreed clinical competencies to work under this protocol.

2. History

Refer to protocol for history taking and clinical documentation.

Use appropriate pain assessment tool as detailed in Appendix A and B according to the age and cognitive function of the patient.

- Document a full history including:

Reference to trust policy on pain management. Assess pain and administer analgesia and record a pain score in the documentation as per patient Group Direction (PGD).

Check for allergies and sensitivities prior to administrating analgesia.

Discuss methods of analgesia with patient, parents and carers.

Consider the use of distraction techniques for younger patients.

Document any refusal of analgesia by the patient.

Re-assess the patient 20 minutes post analgesia and document findings.

Document adherence to Patient Group Directions (PGD)

3. Treatment

<u>Adults – use a stepwise strategy for managing mild moderate pain:</u>

1) paracetamol – maximum dose 1 gram four times a day as PGD

2) ibuprofen – 400mg three times a day. If a non- steroidal anti-inflammatory drug (NSAID) is unsuitable a weak opioid should be used codeine 30-60mg 4-6 hours as PGD.

3) paracetamol and low dose ibuprofen. If patient is unable to tolerate an NSAID, paracetamol can be added to codeine.

4) paracetamol, codeine and / or ibuprofen

Treat the underlying cause of pain. Caution is needed with long-term use of weak opioids as tolerance and dependence can occur.

For the elderly if using a weak opioid commence with lower dose, 30mg codeine phosphate. Consider the use of entonox if appropriate as per PGD

Children – under 16 years of age:

Paracetamol or ibuprofen alone are suitable first-line choices.

Check for compliance

If the child has not responded sufficiently to the appropriate doses of one drug alone alternating paracetamol and ibuprofen should be considered.

Paracetamol is given every 6 hours and ibuprofen every 8 hours.

Administer as PGD.

Moderate to severe pain in adults:

Stat dose oral morphine sulphate in addition to paracetamol and / or ibuprofen according to the PGD

- If oral morphine sulphate is administered monitor vital signs (see Appendix B).
- Review by medical practitioner of refer to secondary care (Emergency Department) as appropriate
- Consider Step 4 of the stepwise strategy as documented above.

If stronger analgesia is required for any patient, refer to a medical practitioner or secondary care.

Moderate to severe pain in children:

Stat dose of oral morphine sulphate in addition to paracetamol and/or ibuprofen as per PGD

If oral morphine sulphate is administered monitor vital signs

Review by medical practitioner or refer to the Emergency Department as appropriate

Ametop gel may be applied as per PGD if venepuncture or cannulation of a child is likely to be required.

4. Discharge Pathway

Assess and document pain score prior to discharge

Ensure patient is issued with appropriate advice sheet (if available) and that patient understands the need to return if symptoms change or worsens.

Discuss home analgesia with patient, parent or carer and advise OTC medication or supply TTO medication as per PGD.

4.1. DOCUMENTATION TO BE COMPLETED

- Clinical treatment record as per Documentation and Record keeping policies. Copy of clinical treatment record to General Practitioner; to be sent to surgery as per Record keeping policy.
- For patients being transferred to secondary care, ensure a copy of the clinical treatment record is sent with patient. A copy will also be sent to surgery in normal manner.

4.2. BEFORE DISCHARGE ENSURE

- Those patients who have been referred for further acute intervention have appropriate transport to meet their needs, all relevant treatment has been prescribed and administered and correct information and documentation is given to the patient.
- The patient understands that if condition deteriorates or they have further concerns they should seek further advice.
- The patient demonstrates understanding of advice given during consultation.
- The patient demonstrates an understanding of how to manage subsequent problems.

5. References

British National Formulary online edition via

https://www.new.medicinescomplete.com/

British National Formulary for Children online edition via

https://www.new.medicinescomplete.com/

Clinical Knowledge Summaries – Analgesia mild to moderate pain (April 2015)

Consent Policy NDHCT (2018)

Clinical Record Keeping Policy NDHCT (2015)

Medicines Policy NDHCT (2018)

Patient Group Direction Policy NDHCT (2016)

Patients at Risk of Deterioration Policy NDHCT (2019)

APPENDIX A – Essential Documentation for All Patients Attending Unit or Centre

Adults Consent
Gain consent to be seen by a nurse practitioner
Gain consent for treatment and sharing information and document.

Clinical Presentation
If unwell assess for:

- Airway
- Breathing
- Circulation
- Disability
- Exposure

Document a full set of observations including neurological observations including Glasgow coma score if applicable.

Record NEWS 2 or PEWS for handover to secondary care or medical practitioner(GP) if applicable.

Document pain score using numeric rating scale. For cognitively impaired patients document any signs of pain (e.g. grimaces or distress). Re document pain score and reassess if analgesia has been administered.

Safeguarding:

- Ask the domestic abuse question, 'do you feel safe at home?'
- Assess for mental capacity and if the person is a vulnerable adult.
- Assess for learning disability and whether patient has a hospital passport in place.
- Assess falls risk. Complete falls referral if applicable.

APPENDIX B – Essential Documentation for All Patients Attending Unit or Centre

Child and Young Persons under 18 Years Old Consent
Gain consent to be seen by a nurse practitioner
Gain consent for treatment and sharing information
Assess and document competence according to Fraser guideline if applicable.
Document name of person(s) accompanying patient

Clinical Presentation

If unwell assess for:

- Airway
- Breathing
- Circulation
- Disability
- Exposure

Record PEWS: if any one parameter is triggered transfer to secondary care or seek advice from medical practitioner.

Use guideline Traffic Light System (NICE) 2013 if applicable.
Use guideline Feverish Illness (NICE) 2013 if applicable.
Document pain score using FLACC, Wong Baker Faces or numeric rating scale.

Safeguarding Children:

- Complete safeguarding children's questions (NICE2003)
- Any injury or bruise on a non-mobile infant or child: follow Safeguarding Children Policy (2018). These children must be reviewed by a Consultant in Emergency Medicine or a Consultant Paediatrician and a MASH (multi agency safeguarding hub) referral must be made.
- Assess for learning disability

DOCUMENT ALL FINDINGS IN THE CLINICAL TREATMENT RECORD AND ACT ON THEM FOLLOWING NDHCT GUIDELINES.

APPENDIX C – Training Competency Form

Protocol for Pain Management in Minor Injury Units

Protocol operational from May 2019 and expires end of May 2022

The registered health professional named below, being employees of Northern Devon Healthcare Trust based at ……. have received training and are competent to operate under this protocol

NAME *(please print)*	PROFESSIONAL TITLE	SIGNATURE	AUTHORISING MANAGER *(please print)*	MANAGER'S SIGNATURE	DATE

Keep original with the authorising manager and send a copy to: Karen Watts, Emergency Department, Northern Devon Healthcare Trust NHS, Raleigh Park, Barnstaple, Devon, EX31 4JB